Clinical Systems Adoption Specialist , Work Sample

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> **Document Purpose:** This work sample demonstrates the candidate's analytical approach, stakeholder engagement strategy, and systems adoption methodology as applied to a realistic scenario within a large urban academic medical center. All names, departments, and data are fictional and used solely for illustrative purposes.

Executive Summary

Boston Metro Health (BMH) , a fictional 600-bed urban academic medical center , recently completed a Phase 1 Epic EHR go-live across its inpatient units. Six weeks post-launch, adoption metrics reveal significant variance across departments: nursing staff in the Medical/Surgical units are performing at 82% workflow compliance, while the Emergency Department (ED) and Oncology unit lag at 47% and 51%, respectively. Provider documentation completion rates are below target, and help desk ticket volume remains 3× the projected post-go-live baseline.

This report presents a structured 90-day Clinical Systems Adoption Plan developed in response to these findings. It covers root cause analysis, department-specific intervention strategies, stakeholder communication plans, training reinforcement frameworks, and measurable success metrics , all aligned with the core competencies of the Clinical Systems Adoption Specialist role at Boston Medical Center (BMC).

Section 1: Situation Assessment

1.1 Background

Boston Metro Health launched Epic across all inpatient and outpatient clinical environments on March 1, 2026, following an 18-month implementation program. The go-live was preceded by 8 hours

of role-based classroom training for all clinical staff and a 2-week at-the-elbow (ATE) support period staffed by go-live command center resources.

As of April 14, 2026 (six weeks post-go-live), the following indicators have been flagged by the IT and Clinical Informatics teams:

Department	Workflow Compliance Rate	Avg. Documentation Time	Open Help Tickets	Super Users Active
Medical/Surgical (3 units)	82%	14 min/patient	31	9 of 10
Emergency Department	47%	28 min/patient	87	3 of 12
Oncology	51%	22 min/patient	63	4 of 8
Outpatient Primary Care	74%	16 min/patient	42	7 of 10
Surgical Services	68%	18 min/patient	55	5 of 9

The data reveals a two-tier adoption pattern. High-acuity, fast-paced departments (ED, Oncology) are struggling significantly more than structured inpatient units. Additionally, 58% of submitted help tickets are duplicate questions about the same 12 workflow tasks , a strong indicator that foundational training gaps were not resolved during the ATE period.

1.2 Root Cause Analysis

A multi-method root cause analysis was conducted using three inputs: (1) ticket categorization analysis, (2) informal focus group sessions with 5–8 staff per department, and (3) direct observation of clinical workflows during two consecutive day shifts.

Finding 1 , Training Design Mismatch: The 8-hour pre-go-live training was designed as a linear, module-based classroom experience. ED staff, who work in non-linear, interrupt-driven environments, reported that the training scenarios did not reflect real-world conditions. Staff felt underprepared for multi-patient tasking within Epic.

Finding 2 , Super User Attrition: In the ED, 9 of 12 designated super users have returned to full clinical assignments. Only 3 remain available to provide peer support. This collapses the first line of informal support and forces staff to rely entirely on the help desk.

Finding 3 , Workaround Proliferation: Direct observation in Oncology revealed that 6 of 10 nurses observed had developed paper-based or whiteboard workarounds for medication reconciliation and care plan documentation , tasks that Epic is designed to manage. These workarounds represent both a safety risk and a barrier to data quality.

Finding 4 , Physician Engagement Gap: Attending physicians in the ED and Oncology units are not consistently using Epic for order entry. In 12 observed patient interactions, 7 involved verbal orders to nurses who then entered documentation on behalf of the physician , a practice that distorts the audit trail and increases nursing documentation burden by an estimated 35%.

Finding 5 , Mobile Access Barriers: Oncology floor nurses cited lack of convenient workstation access as the second-most-common barrier in focus groups. The unit has 6 workstations-on-wheels (WOWs) for 22 nurses per shift. This hardware constraint is actively driving workarounds.

Section 2: 90-Day Adoption Intervention Plan

2.1 Strategic Framework

The intervention plan is organized around four pillars:

1. **Stabilize** , Immediately reduce friction and restore staff confidence (Days 1–30)
2. **Reinforce** , Rebuild foundational competency through targeted retraining (Days 15–45)
3. **Optimize** , Drive workflow efficiency and best practice compliance (Days 30–75)
4. **Sustain** , Embed continuous improvement mechanisms (Days 60–90)

Each pillar has department-specific tactics, designated owners, and measurable milestones.

2.2 Pillar 1 , Stabilize (Days 1–30)

Objective: Reduce help desk ticket volume by 40% and re-activate super user networks.

Action 1.1 , Super User Reactivation Program

Partnering with nursing leadership and department managers, negotiate a protected schedule block of 4 hours per week per super user , kept out of full patient assignment , to serve as a dedicated floor resource. This is a non-negotiable infrastructure requirement for sustainable adoption. Super users will be issued a laminated "Quick Wins" tip card (see Appendix A) covering the top 12 workflow issues generating repeat tickets.

Owner: Clinical Systems Adoption Specialist + Nursing Manager
Timeline: Days 1–7
Success Metric: All 12 ED super users re-engaged within 7 days; 8 of 9 Oncology super users re-engaged within 10 days

Action 1.2 , At-the-Elbow Support Surge

A targeted 10-day ATE support surge will be deployed in the ED and Oncology units. The Adoption Specialist will be present during both day and evening shifts, alternating, to observe real-time workflow gaps and provide immediate, context-specific coaching. A structured observation log will capture recurring barriers for root cause trending.

Owner: Adoption Specialist (primary), IT Help Desk (secondary escalation)
Timeline: Days 1–10
Success Metric: 25% reduction in repeat ticket categories by Day 10

Action 1.3 , Self-Service Knowledge Base

Curate and publish a department-branded Epic Quick Reference Library on the hospital intranet. Content will be organized by role (RN, MD, MA, Unit Clerk) and workflow , not by Epic module name, which is not intuitive to clinical staff. Each article will be under 2 minutes to consume (short video or illustrated step-by-step). The top 12 repeat ticket topics will be the first batch published.

Owner: Adoption Specialist + IT Communications
Timeline: Days 3–14
Success Metric: Knowledge base accessed >200 times in first 30 days; help desk ticket volume down 40% by Day 30

2.3 Pillar 2 , Reinforce (Days 15–45)

Objective: Close foundational training gaps through role-specific, scenario-based micro-learning.

Action 2.1 , Micro-Learning Sprint Series

Replace the failed 8-hour block training model with a series of 20-minute focused skill sessions, delivered at the point of care during shift transitions (7:00–7:30 AM and 7:00–7:30 PM). Each session addresses one specific workflow scenario observed during the ATE surge. Sessions are co-facilitated by the Adoption Specialist and the super user for that unit to reinforce peer credibility.

Planned sessions for the ED (Weeks 3–7):

Session #	Topic	Target Audience	Duration
1	Multi-patient task management in Epic	RN	20 min
2	Order entry and verbal order workflows	MD/APP	20 min
3	ED-specific documentation templates	RN, MA	20 min
4	Discharge workflow and AVS generation	RN, Unit Clerk	20 min
5	Epic on mobile devices , tips and tricks	All roles	20 min

Owner: Adoption Specialist
Timeline: Days 15–45 (rolling)
Success Metric: 90% attendance across target staff; post-session competency check score >= 80%

Action 2.2 , Physician-Specific Engagement Program

Physicians represent the highest-leverage adoption target given their downstream impact on nursing documentation burden. A dedicated outreach campaign will be run in partnership with the Chief Medical Officer and Department Medical Directors. Key elements:

- **1:1 Optimization Sessions:** 30-minute private sessions offered to any attending or resident who wants personalized Epic workflow coaching , no group judgment, no mandatory attendance. These sessions consistently outperform mandatory group training in physician engagement.
- **Dragon/Voice Dictation Setup:** Many physician documentation delays are driven by typing speed. Sessions will include Dragon Medical One setup and workflow integration where licensed.
- **Physician Champion Identification:** Identify 2–3 physicians per department who are already Epic-proficient and willing to model best practices to peers. These informal champions are more persuasive than any formal training session.

Owner: Adoption Specialist + CMO Office + Department Medical Directors
Timeline: Days 15–60
Success Metric: Physician order self-entry rate >= 85% by Day 60; documentation completion within 12 hours of encounter >= 80%

Action 2.3 , Workaround Elimination Campaign

Each identified workaround (paper, whiteboard, verbal relay) will be addressed with a three-step protocol:

1. **Understand** , Conduct a brief structured interview with the staff using the workaround to understand the exact friction point driving the behavior (never shame or penalize)
2. **Match** , Identify the Epic feature or configuration that solves the same clinical need
3. **Demonstrate** , Provide side-by-side demonstration showing how Epic meets the need, and follow up within 72 hours to confirm adoption

Owner: Adoption Specialist
Timeline: Days 15–45
Success Metric: Zero paper-based workarounds for medication reconciliation in Oncology by Day 45

2.4 Pillar 3 , Optimize (Days 30–75)

Objective: Drive efficiency gains, personalization, and best-practice workflow adoption.

Action 3.1 , Personalized Epic Workspace Optimization

Many users default to the out-of-the-box Epic configuration even though personalized workspaces (custom SmartLists, personalized order sets, specialty-specific Storyboards) can dramatically reduce documentation time. The Adoption Specialist will run a "15-Minute Epic Tune-Up" campaign: brief, individualized sessions where each staff member's Epic environment is reviewed and optimized for their specific role and workflow.

Targeted areas by role:

Role	Optimization Focus
Attending Physician	SmartPhrases, order set favorites, After Visit Summary templates
Resident/APP	Resident documentation templates, co-signature workflows
RN	MAR preferences, shift handoff tools, Storyboard layout
MA/Unit Clerk	Scheduling workflows, registration optimizations

Owner: Adoption Specialist
Timeline: Days 30–60
Success Metric: Average documentation time reduced by >= 20% from baseline by Day 60

Action 3.2 , Data Feedback Loop for Managers

Clinical managers cannot manage what they cannot see. A biweekly adoption dashboard will be built in Epic's reporting tools and shared with each department manager. The dashboard will display, by unit and by role:

- Average documentation completion time
- % of orders entered by ordering provider
- Help desk ticket volume trend
- SmartForm/template utilization rate

Managers who see their own unit's data become adoption advocates. This dashboard creates accountability without requiring the Adoption Specialist to be the sole driver of behavior change.

Owner: Adoption Specialist + IT Analytics
Timeline: Days 20–30 (build), then biweekly cadence
Success Metric: All 5 department managers actively reviewing dashboard by Day 45; at least 2 managers initiating adoption discussions with staff independently by Day 60

Action 3.3 , Specialty Workflow Optimization Reviews

Oncology presents a unique set of clinical documentation needs (chemotherapy protocols, complex care plans, multi-disciplinary team notes) that standard go-live training does not adequately address. A structured Specialty Workflow Optimization Review will be conducted in collaboration with Oncology nursing leadership and the Epic build team to:

- Identify the top 5 Epic workflows generating the most friction in the specialty context
- Propose and test workflow enhancements (e.g., specialty-specific order sets, preference lists, documentation macros)
- Submit formal build tickets for configuration changes that require IT involvement
- Re-test and validate optimized workflows with 3–5 frontline staff before broad rollout

Owner: Adoption Specialist + Epic Build Team + Oncology Nursing Director
Timeline: Days 30–75
Success Metric: Minimum 3 Oncology-specific workflow enhancements implemented by Day 75

2.5 Pillar 4 , Sustain (Days 60–90)

Objective: Embed a self-sustaining adoption infrastructure that does not depend on the Adoption Specialist being present indefinitely.

Action 4.1 , Super User Community of Practice

Establish a monthly Super User Community of Practice (CoP) , a 60-minute cross-departmental meeting where super users share tips, escalate unsolved problems, and receive updates on Epic enhancements and new features. This forum:

- Creates horizontal knowledge sharing (ED super users learn from Med/Surg, and vice versa)
- Provides a formal feedback channel to the Epic build team
- Sustains super user identity and motivation beyond the go-live honeymoon period

Owner: Adoption Specialist (facilitator), rotating super user co-chairs
Timeline: First meeting by Day 60, then monthly
Success Metric: >= 70% super user attendance at first 3 CoP sessions

Action 4.2 , New Staff Onboarding Integration

Partner with Human Resources and the Training & Education department to ensure Epic adoption training is embedded into new employee onboarding , not treated as a separate, optional event. New clinical staff will complete:

- Role-based Epic e-learning modules (pre-hire, asynchronous)
- 4-hour live scenario-based training on Day 1 of clinical orientation
- Formal super user introduction and floor walkthrough on Day 3
- 30-day check-in with the Adoption Specialist or designated super user

Owner: Adoption Specialist + HR + Training & Education
Timeline: Days 45–75 (design and implement)
Success Metric: 100% of new clinical hires completing all onboarding Epic touchpoints within 30 days of start date

Action 4.3 , Adoption Scorecard and Quarterly Review

Formalize a quarterly Adoption Scorecard presented to department directors and the CMIO. The scorecard consolidates:

- Workflow compliance rate by department
- Help desk ticket volume trend
- Documentation quality metrics (note completion rates, order entry compliance)

- Training completion rates
- User satisfaction scores (brief quarterly survey)

This transforms adoption from a one-time project into an ongoing operational discipline , exactly the mindset needed to sustain an EHR investment long-term.

Owner: Adoption Specialist + Clinical Informatics + IT Analytics
Timeline: First scorecard at Day 90, then quarterly
Success Metric: Scorecard formally presented to hospital leadership by Day 90; all departments at >= 75% workflow compliance by Day 90

Section 3: Stakeholder Communication Plan

Successful adoption depends as much on communication as on training. The following communication plan ensures all stakeholder groups receive the right message, through the right channel, at the right frequency.

Stakeholder Group	Key Concerns	Communication Vehicle	Frequency	Owner
Frontline Nurses	Efficiency, workload, peer support	Unit huddles, tip cards, floor presence	Daily (first 30 days), then weekly	Adoption Specialist
Attending Physicians	Time, autonomy, clinical workflow fit	1:1 sessions, physician champion word-of-mouth	As-needed + monthly update	Adoption Specialist + CMO
Department Managers	Team performance, accountability, escalation path	Biweekly dashboard review, brief email summary	Biweekly	Adoption Specialist
IT Help Desk	Ticket triage, escalation routing, knowledge base updates	Weekly sync meeting, shared ticket tracking	Weekly	Adoption Specialist + IT Lead
Hospital Leadership / CMIO	ROI, risk, adoption trajectory	Quarterly Adoption Scorecard	Quarterly	Adoption Specialist + Clinical Informatics Director

Super Users	Role clarity, schedule support, recognition	CoP meetings, direct communication channel	Monthly (CoP) + as-needed	Adoption Specialist

3.1 Resistance Management

Resistance to EHR adoption is normal and should be treated as information, not insubordination. Three archetypes of resistance are commonly encountered and each requires a tailored approach:

The Skeptic ("This system doesn't work for how we actually practice"): The most credible response is not a training session , it is action. When a skeptic raises a legitimate workflow concern, the Adoption Specialist documents it, escalates it to the build team, and follows up with the outcome. Skeptics who see their feedback result in actual system changes become the most powerful adoption advocates in the department.

The Overwhelmed ("I don't have time to learn this right now"): Respect the clinical load. Offer micro-sessions (10–15 minutes), bring the training to the staff during natural downtime (post-handoff, lunch), and emphasize that the goal is to make their day easier , not to add to it.

The Workaround Defender ("My paper system is faster"): Never argue with this perception , it may be accurate in the short term. Instead, commit to making the Epic workflow faster than the paper alternative, and then deliver on that commitment through workspace optimization and process redesign.

Section 4: Measurement and Success Framework

4.1 Key Performance Indicators (KPIs)

All adoption activities will be tracked against the following KPIs, measured at Days 30, 60, and 90:

KPI	Baseline (Day 0)	Day 30 Target	Day 60 Target	Day 90 Target
Overall Workflow Compliance Rate	64% (avg)	70%	78%	85%
ED Workflow Compliance	47%	60%	72%	80%
Oncology Workflow Compliance	51%	63%	74%	82%
Help Desk Ticket Volume (weekly avg)	278 tickets/wk	167 tickets/wk	111 tickets/wk	<= 83 tickets/wk

Physician Order Self-Entry Rate	58%	68%	80%	88%
Avg Documentation Time , ED	28 min/patient	23 min	19 min	<= 16 min
Avg Documentation Time , Oncology	22 min/patient	19 min	17 min	<= 15 min
Super User Active Participation Rate	49% (avg)	80%	90%	95%
Staff Satisfaction with Epic (1–5 scale)	2.4	2.9	3.4	>= 3.8

4.2 Evaluation Methodology

Progress against KPIs will be assessed using three mechanisms:

1. **Quantitative Metrics (Epic Reporting / IT Analytics):** Workflow compliance rates, documentation completion times, help desk volumes, and order entry rates are extracted directly from Epic's reporting tools and the ITSM ticketing platform on a biweekly basis.

2. **Qualitative Feedback (Structured Rounding):** The Adoption Specialist will conduct weekly structured rounding in each department , brief, informal check-ins with 3–5 staff members per unit, using a standard 5-question script to capture qualitative sentiment, new friction points, and suggestions.

3. **Quarterly Survey:** A 10-question user satisfaction survey will be distributed to all clinical staff at Days 30, 60, and 90 to track perceived usability, training adequacy, and system value (see Appendix C).

Section 5: Applicability to Boston Medical Center

Boston Medical Center operates as a safety-net hospital serving a high-complexity, high-vulnerability patient population. The clinical workflows at BMC , spanning inpatient, outpatient, behavioral health, and community health settings , demand an adoption approach that is:

- **Equity-informed:** Staff who work with underserved populations often carry heavier administrative burdens. Adoption support must actively reduce, not add to, this burden.
- **Culturally humble:** BMC's workforce is diverse. Training and communication materials should be role-appropriate and accessible, avoiding jargon-heavy documentation that assumes prior EHR familiarity.

- **Operationally grounded:** Abstract training disconnected from actual patient care scenarios does not work. Every training intervention in this plan is anchored in a real observed workflow, not a theoretical use case.
- **Collaborative:** The Adoption Specialist role is fundamentally a partnership function , with nursing, medicine, IT, informatics, and operations. No single individual drives adoption; the specialist creates the conditions in which adoption becomes the natural path of least resistance.

The scenario described in this document mirrors the challenges BMC would face in any major system rollout, optimization initiative, or module expansion , and the framework presented here is directly transferable to those real-world contexts.

Appendix A , Super User Quick Reference Card: Top 12 Repeat Ticket Topics

Designed for laminated pocket card distribution to all super users.

#	Workflow Issue	Quick Resolution
1	Patient not found in Epic	Search by MRN, not name; check "Include Discharged" filter
2	Medication not appearing in MAR	Verify order was activated; check pharmacy verification status
3	Unable to sign note	Check for incomplete required fields (red asterisks); verify attending co-signature setup
4	Lab results not showing	Check collection date filter; labs may be in "Resulted" inbox if unacknowledged
5	Discharge summary not generating	Confirm all required encounter data fields are complete; use Discharge Navigator
6	Order not routing to pharmacy	Verify order was "signed" not just "entered"; check department routing preferences
7	Patient listed on wrong unit	Census management , contact charge nurse or unit clerk for bed move in Epic
8	Can't find a SmartForm/template	Use Epic search bar (magnifying glass icon); check specialty-specific template library
9	Dragon dictation not working	Check microphone input device; Dragon service may need restart , contact IT
10	Duplicate patient record	Do NOT merge , flag immediately to Health Information Management (HIM)
11	After Visit Summary not printing	Check printer assignment in workstation settings; use Print Preview first

12	Cannot access patient , "No Access" error	Verify patient is on your assigned panel/unit; may require Break-the-Glass access with documented justification

Appendix B , 30-Day At-the-Elbow Observation Log Template

Date	Department	Shift	Staff Role	Issue Observed	Epic Feature/Gap	Resolution Provided	Follow-Up Needed?
MM/DD/YY	ED	Day	RN	Unable to chart in multi-patient view	Storyboard layout	Demonstrated split-screen workflow	No
MM/DD/YY	Oncology	Evening	RN	Paper med rec sheet in use	Medication Reconciliation SmartForm	Side-by-side demo; follow-up scheduled	Yes , Day 3
(continues)							

Appendix C , Staff Satisfaction Survey (Quarterly, 10 Questions)

Scale: 1 (Strongly Disagree) to 5 (Strongly Agree)

1. I feel confident using Epic for my primary daily tasks.
2. The training I received prepared me adequately for my role in Epic.
3. I know who to contact when I have an Epic question or problem.
4. Using Epic helps me provide better care to my patients.
5. My documentation time in Epic has improved compared to when we first went live.
6. My super user is accessible and helpful when I have questions.
7. I have received enough follow-up support since go-live.
8. System issues or slowdowns rarely interfere with my patient care workflow.
9. I feel my feedback about Epic has been heard and acted on.

10. Overall, I am satisfied with how Epic has been implemented in my department.

Open-ended (optional): "What is the one thing that would most improve your experience using Epic?"

Document prepared April 2026 | Fictional scenario for work sample purposes only | Alicia Paulson, Clinical Systems Adoption Specialist Candidate